

TEST QUESTIONS FOR CHATBOT

Based on NICE Type 2 Diabetes Guideline (NG28)

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SECTION 1: INDIVIDUALISED CARE (1.1)

1. What approach to diabetes care should be adopted according to recommendation 1.1.1?
2. How often should a person's needs and circumstances be reassessed? (1.1.2)
3. What should be taken into account when planning care for adults with visual impairment? (1.1.3)

SECTION 2: STRUCTURED EDUCATION (1.2)

4. When should structured education be offered to adults with type 2 diabetes? (1.2.1)
5. List five criteria that a structured education programme must meet. (1.2.2)
6. What is the preferred option for delivering education programmes? (1.2.4)
7. How should education programmes address cultural, linguistic, cognitive and literacy needs? (1.2.5)

SECTION 3: DIETARY ADVICE (1.3)

8. Who should provide dietary advice to adults with type 2 diabetes? (1.3.1)

9. What healthy eating advice should adults with type 2 diabetes be encouraged to follow? (1.3.3)

10. What should be integrated with dietary advice according to recommendation 1.3.5?

11. What should be discouraged regarding foods marketed specifically for people with diabetes? (1.3.8)

12. What meal planning system should be implemented when adults with type 2 diabetes are admitted to hospital? (1.3.9)

SECTION 4: HbA1c MEASUREMENT AND TARGETS (1.5)

13. How often should HbA1c levels be measured in adults with type 2 diabetes? (1.5.1)

14. What methods should be used if HbA1c monitoring is invalid due to disturbed erythrocyte turnover? (1.5.3)

15. What HbA1c target should be supported for adults managed by diet alone or with non-hypoglycaemic medication? (1.5.7)

16. What HbA1c target should be supported for adults on a medicine associated with hypoglycaemia? (1.5.7)

17. At what HbA1c level should intensification of medicines be considered? (1.5.8)

18. When should a target HbA1c level be relaxed? (1.5.9)

SECTION 5: SELF-MONITORING OF CAPILLARY BLOOD GLUCOSE (1.6)

19. What should be taken into account when offering self-monitoring of capillary blood glucose? (1.6.1)

20. When should self-monitoring of capillary blood glucose NOT be routinely offered? (1.6.2)

21. List four things that should be included in a structured assessment of self-monitoring skills. (1.6.5)

SECTION 6: CONTINUOUS GLUCOSE MONITORING (1.7)

22. When should intermittently scanned CGM (isCGM) be offered to adults on multiple daily insulin injections? (1.7.1)

23. When should isCGM be offered to adults who need help from a care worker? (1.7.2)

24. When can real-time CGM (rtCGM) be considered as an alternative? (1.7.3)

25. Who should provide CGM according to recommendation 1.7.4?

26. Why will adults using CGM still need to take capillary blood glucose measurements? (1.7.5)

SECTION 7: INITIAL MEDICINES – NO RELEVANT COMORBIDITY (1.13)

27. What initial medication combination should be offered to adults with no relevant comorbidity? (1.13.1)

28. What should be offered if metformin is contraindicated or not tolerated? (1.13.2)

SECTION 8: INITIAL MEDICINES – HEART FAILURE (1.14)

29. What should be offered to adults with type 2 diabetes and heart failure? (1.14.1)

30. What should be offered if metformin is contraindicated or not tolerated in people with heart failure? (1.14.2)

SECTION 9: INITIAL MEDICINES – ATHEROSCLEROTIC CARDIOVASCULAR DISEASE (1.15)

31. What three medications should be offered to adults with atherosclerotic cardiovascular disease? (1.15.1)
32. What is the dose recommendation for subcutaneous semaglutide (Ozempic)? (1.15.1)
33. What should be offered if metformin is contraindicated or not tolerated? (1.15.2)

SECTION 10: INITIAL MEDICINES – EARLY ONSET TYPE 2 DIABETES (1.16)

34. What is the definition of early onset type 2 diabetes? (see Terms Used)
35. What should be offered to adults with early onset type 2 diabetes? (1.16.1)
36. What two options should be considered for addition in early onset type 2 diabetes? (1.16.1)
37. What should be offered if metformin is contraindicated or not tolerated? (1.16.2)

SECTION 11: INITIAL MEDICINES – PEOPLE LIVING WITH OBESITY (1.17)

38. What should be offered to adults with type 2 diabetes who are living with obesity? (1.17.1)
39. What should be offered if metformin is contraindicated or not tolerated? (1.17.2)

SECTION 12: INITIAL MEDICINES – CHRONIC KIDNEY DISEASE (1.18)

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40. What should be offered to adults with an eGFR above 30 ml/min/1.73m²? (1.18.1)
41. What should be offered to adults with an eGFR between 20 and 30 ml/min/1.73m²? (1.18.2)
42. What should be considered for adults with an eGFR below 20 ml/min/1.73m²? (1.18.3)

SECTION 13: INITIAL MEDICINES – FRAILITY (1.19)

43. What should be offered to adults with type 2 diabetes and frailty? (1.19.1)
44. When should an SGLT-2 inhibitor be offered to someone with frailty? (1.19.1)
45. What should be considered if metformin is contraindicated and frailty puts the person at risk? (1.19.2)

SECTION 14: INTRODUCING MEDICINES AND PREVENTING DKA (1.20–1.21)

46. How should medicines be introduced according to recommendation 1.20.1?
47. What is the recommended order of introducing medicines when starting initial therapy? (1.20.2)
48. What risk factors for diabetic ketoacidosis should be checked before starting an SGLT-2 inhibitor? (1.21.1)
49. What advice should be given regarding very low carbohydrate or ketogenic diets while taking SGLT-2 inhibitors? (1.21.3)

SECTION 15: REVIEWING MEDICINES (1.22–1.24)

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50. What should be considered when reviewing treatments? (1.22.1–1.22.2)
51. Should SGLT-2 inhibitors be continued even if they don't help reach glycaemic targets? (1.24.2)
52. When should GLP-1 receptor agonists or tirzepatide be stopped? (1.24.3, 1.24.4)
53. Should a GLP-1 receptor agonist and a DPP-4 inhibitor be offered together? (1.24.6)

SECTION 16: FURTHER MEDICATION (1.25–1.31)

54. What should be offered as further medication for people with no relevant comorbidity? (1.25.1)
55. What should be offered as further medication for people with heart failure? (1.26.1)
56. What should be offered to people with atherosclerotic cardiovascular disease who need further medicines? (1.27.1–1.27.2)
57. What considerations apply when choosing treatments for people with frailty? (1.31.2)

SECTION 17: INSULIN-BASED TREATMENTS (1.32–1.36)

58. What should a structured education programme for starting insulin include? (1.32.1)
59. What should be continued and what should be stopped when initiating insulin? (1.32.2)
60. What type of insulin should be offered as initial insulin therapy? (1.33.1)
61. When should combined basal and short/rapid acting insulin be considered? (1.33.2)

SECTION 18: COMPLICATIONS (1.37–1.45)

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62. What advice should be given about periodontitis at annual reviews? (1.37.1)
63. What should be considered in adults with erratic blood glucose control or unexplained gastric bloating? (1.38.1)
64. What treatment options are available for vomiting caused by gastroparesis? (1.38.2–1.38.3)
65. What should be offered as part of the annual review regarding erectile dysfunction? (1.42.1)
66. What should NOT be offered to adults with type 2 diabetes without cardiovascular disease? (1.45.1)

SECTION 19: SCENARIO-BASED QUESTIONS

67. A 65-year-old patient with type 2 diabetes, heart failure (ejection fraction 35%), and an eGFR of 45 ml/min/1.73m² is starting treatment. What initial medicines would you recommend? (1.14.1 & 1.18.1)
68. A 35-year-old patient is newly diagnosed with type 2 diabetes and has a BMI of 38 kg/m². What initial treatment would you offer? (1.16.1 & 1.17.1)
69. A 70-year-old patient with type 2 diabetes and frailty is starting treatment. What factors would influence your decision about prescribing an SGLT-2 inhibitor? (1.19.1)
70. A patient on metformin and an SGLT-2 inhibitor has an HbA1c of 65 mmol/mol (8.1%) after 6 months. No relevant comorbidities. What further medication would you consider? (1.25.1)
71. A patient with atherosclerotic cardiovascular disease is already taking metformin and an SGLT-2 inhibitor but HbA1c remains at 68 mmol/mol (8.4%). What would you add? (1.15.1)
72. A patient with early onset type 2 diabetes is taking metformin and an SGLT-2 inhibitor but HbA1c is 72 mmol/mol (8.7%). What two options could be considered? (1.16.1)

73. A patient with chronic kidney disease has an eGFR of 25 mL/min/1.73m² and HbA1c is 69 mmol/mol (8.5%). What treatment would you recommend? (1.18.2)

74. A patient on a GLP-1 receptor agonist has lost significant weight and their BMI has dropped to 17.5 kg/m². What should you do? (1.24.3)

75. A patient with frailty on metformin has an HbA1c of 70 mmol/mol (8.6%). What further medication would you consider? (1.31.1)

76. A patient is starting insulin therapy. What education should be provided? (1.32.1)

SECTION 20: EVIDENCE AND RATIONALE QUESTIONS

77. According to the committee's rationale, what is the evidence base for recommending modified-release metformin over standard-release? (see rationale 1.13–1.16)

78. Why did the committee recommend SGLT-2 inhibitors for people with no relevant comorbidities despite the risk of adverse events? (see rationale 1.13)

79. What evidence supported the recommendation of subcutaneous semaglutide for people with atherosclerotic cardiovascular disease? (see rationale 1.15)

80. Why are GLP-1 receptor agonists and tirzepatide recommended for consideration in early onset type 2 diabetes? (see rationale 1.16)

81. Why did the committee not recommend GLP-1 receptor agonists for people living with obesity as initial treatment? (see rationale 1.17)

82. What is the rationale for recommending DPP-4 inhibitors for people with an eGFR between 20–30 mL/min/1.73m²? (see rationale 1.18)

83. Why are SGLT-2 inhibitors not recommended for all people with frailty? (see rationale 1.19)

84. What is the evidence base for the recommendation regarding periodontitis? (see rationale 1.37)

SECTION 21: TERMINOLOGY QUESTIONS

- 85. Define "atherosclerotic cardiovascular disease" as used in this guideline.
 - 86. What is "early onset type 2 diabetes"?
 - 87. Define "recurrent hypoglycaemia" and "severe hypoglycaemia".
 - 88. What is meant by "maximum tolerated dose"?
 - 89. Define "no relevant comorbidity" as used in this guideline.
 - 90. What is the definition of a "very low carbohydrate and ketogenic diet"?
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SECTION 22: RESEARCH RECOMMENDATIONS QUESTIONS

- 91. What research is recommended regarding treatment strategies for people with type 2 diabetes and frailty?
 - 92. What research is recommended regarding access to SGLT-2 inhibitors?
 - 93. What research is recommended regarding treatments for people with early onset type 2 diabetes?
 - 94. What research is recommended regarding the effects of stopping or switching medicines?
 - 95. What research is recommended regarding continuous glucose monitoring?
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SECTION 23: POLICY AND IMPLEMENTATION QUESTIONS

- 96. What is the responsibility of local commissioners and providers regarding this guideline?

97. How should healthcare professionals report adverse events related to medicines or medical devices?

98. What does the guideline say about addressing inequalities in access to SGLT-2 inhibitors? (1.12)

99. How should healthcare professionals address inequalities in CGM access and uptake? (1.7.10)

100. What is the recommended approach for sick day rules? (1.10)

SECTION 24: INTEGRATION QUESTIONS

101. Which other NICE guidelines should be read alongside this guideline?

102. What is the relationship between this guideline and the NHS Type 2 Diabetes Path to Remission Programme?

103. What guidance is provided regarding hypertension in people with type 2 diabetes?

104. What guidance is provided regarding diabetic foot problems?

105. What guidance is provided regarding eye disease and diabetic retinopathy?

SECTION 25: CRITICAL THINKING QUESTIONS

106. Why might a healthcare professional choose modified-release metformin over standard-release metformin?

107. What factors should be considered when deciding whether to prescribe an SGLT-2 inhibitor to a frail patient?

108. How would you approach shared decision-making when discussing medication options with a patient?

109. Why is it important to consider cardiovascular and renal protection beyond just glycaemic control?

110. How might the recommendations in this guideline help address health inequalities?

END OF QUESTIONS
